

HOLD

Price \$455.46 (intraday, 3/25/26)

FLASH NOTE**MAZE Data Confirms APOL1 Inhibition Works in FSGS, But Less Benefits Observed for Broader AMKD****Summary**

Today, MAZE reported positive ph2 data for their APOL1 inhibitor in APOL1-mediated kidney disease (AMKD), with relevant implications for VRTX's inaxaplin, also an APOL1 inhibitor, which is currently in the pivotal ph2/3 AMPLITUDE study for AMKD (excluding diabetics). Overall, the MAZE data confirms (1) APOL1 inhibition does show efficacy in the FSGS subgroup [62% uACR reduction], (2) improvement is less prominent in broader non-diabetic AMKD patients [49% uACR reduction], where the FSGS subgroup is doing the heavy lifting--only one of three non-diabetic patients without FSGS actually benefited from drug so far, and (3) diabetic AMKD patients also showed inconsistent benefit. The question for inaxaplin going into pivotal AMKD data is whether the very promising early FSGS results are extrapolatable to a more heterogeneous population (APOL1 with or without FSGS). With caveats that the MAZE molecule is different and their study is small, we still see this as an open question.

Key Points

MAZE data showed clearest benefits in the FSGS subgroup, which confirms VRTX's findings from ph2a. However, benefits in other AMKD populations were questionable. MZE829 is a once-daily oral small molecule that was evaluated in a ph2 basket study including AMKD patients with or without diabetes. The study enrolled 14 AMKD patients, of which 12 (n=4 non-diabetic FSGS, n=3 non-diabetic non-FSGS, n=5 diabetic) were included in the efficacy analysis after excluding patients not meeting the compliance threshold. A 36% mean reduction in UACR was observed across the full efficacy dataset, with most prominent benefits observed in the FSGS subgroup with 62% uACR improvement. MAZE argues that their efficacy in the FSGS subgroup exceeds VRTX's inaxaplin (59% uPCR reduction @ 12 weeks vs 48% @ 13 weeks) due to MAZE's dual mechanism--MZE-829 inhibits both APOL1 channel formation and function, while there's an argument that inaxaplin targets only the latter. However, we still see this mechanistic thesis as a hypothesis, and we're cautious on these cross-trial comparisons given the small Ns and limited clinical data so far. More interestingly, though, benefit was very inconsistent in the non-FSGS populations: only one of the three non-diabetic, non-FSGS patients and two of the five in the diabetic subgroup saw clinically meaningful improvement in UACR ($\geq 30\%$). This has relevant implications for VRTX, who decided to expand from APOL1-FSGS patients in their ph2a for inaxaplin to a broader, non-diabetic AMKD population for their ph2/3 AMPLITUDE study. There are some literature that suggests this strategy is not without risk. For example, one paper describes that the odds-ratio for APOL1 mutations within FSGS is very high (~17x) compared to some of the other more prevalent AMKD subtypes (e.g., hypertension-associated ESRD, non-diabetic CKD). This raises the question of whether these patients have other non-APOL1 factors that are driving kidney injury that could put a ceiling on the efficacy driven by APOL1 inhibition alone. To be fair, both MAZE ph2 and VRTX's ph2a were small and short studies, and benefits may accrue with longer duration of treatment. KOLs have also been very supportive of the APOL1 approach for APOL1-related kidney diseases broadly--but again, this hypothesis (i.e. how well do these drugs work in non-FSGS?) is something that the pivotal study will need to derisk and, in our view, the answer remains TBD.

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Investment Thesis

We rate Vertex shares Hold. Vertex has revolutionized the treatment of CF with its “CFTR modulators” – a new class of small molecule drugs that seek to enhance the expression and activity of the missing protein that is at the epicenter of CF disease. Our thesis and valuation for the stock are largely predicated on Vertex’s high barriers to entry in CF, which were recently heightened via the approval of Trikafta. We think these barriers are strong and durable as we don’t see any worrisome competitors in VRTX’s target market, and will protect a franchise we expect to grow to over \$9B in the midterm. However, one of the main questions for the stock is “what’s the next CF?,” and from our work, we think the current pipeline is intriguing, but it’s unclear to us whether drug candidates like VX-147 (APOL1-FSGS), VX-880 (T1D cell replacement) or CTX001 (gene editing for sickle cell/beta thal) address market opportunities that are meaningful enough to be narrative changing. We are somewhat more positive on the upside potential of VRTX’s chronic pain franchise, though this remains early and still somewhat risky. Beyond the pipeline, we expect VRTX to do more business development, and potentially even a bigger deal(s) at some point, given its cash position. Even still, we view VRTX as a high-quality large-cap biotech as its CF portfolio is one of the more competitively insulated blockbuster franchises – with strong IP – across the biotech/pharma space.

Target Price Methodology/Risks

We derive a \$466/share target price for Vertex using a DCF analysis. Our model extends to 2038 and accounts for potential out-year competition/CF business erosion over the long term. Our model employs an 8.5% discount rate, in the range of other large-cap biotechnology stocks in our coverage universe. We also use a 1% terminal growth rate, crediting the longevity of the CF franchise and the potential for pipeline successes.

Risks to our target price include clinical risks, regulatory risks, commercial risks, manufacturing risks, and/or financing risks. The success of Vertex’s Triple combination is essential to our thesis, and any setbacks with this program could impact the stock and our target price significantly.

Company Description

Vertex Pharmaceuticals Incorporated is a large cap biotechnology company that is focused on the development and commercialization of breakthrough medicines for the treatment of Cystic Fibrosis (CF) and other rare genetic diseases. Vertex has revolutionized the treatment of CF with its “CFTR modulators” – a new class of small molecule drugs that seek to enhance the expression and activity of the missing protein that is at the epicenter of CF disease. Vertex has long discussed the idea of diversifying away from CF and may leverage in-house capabilities and/or business development to bolster its pipeline.

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Vertex Pharmaceuticals Incorporated (VRTX) as of March 24, 2026 (in USD)



*Represents the value(s) that changed.

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